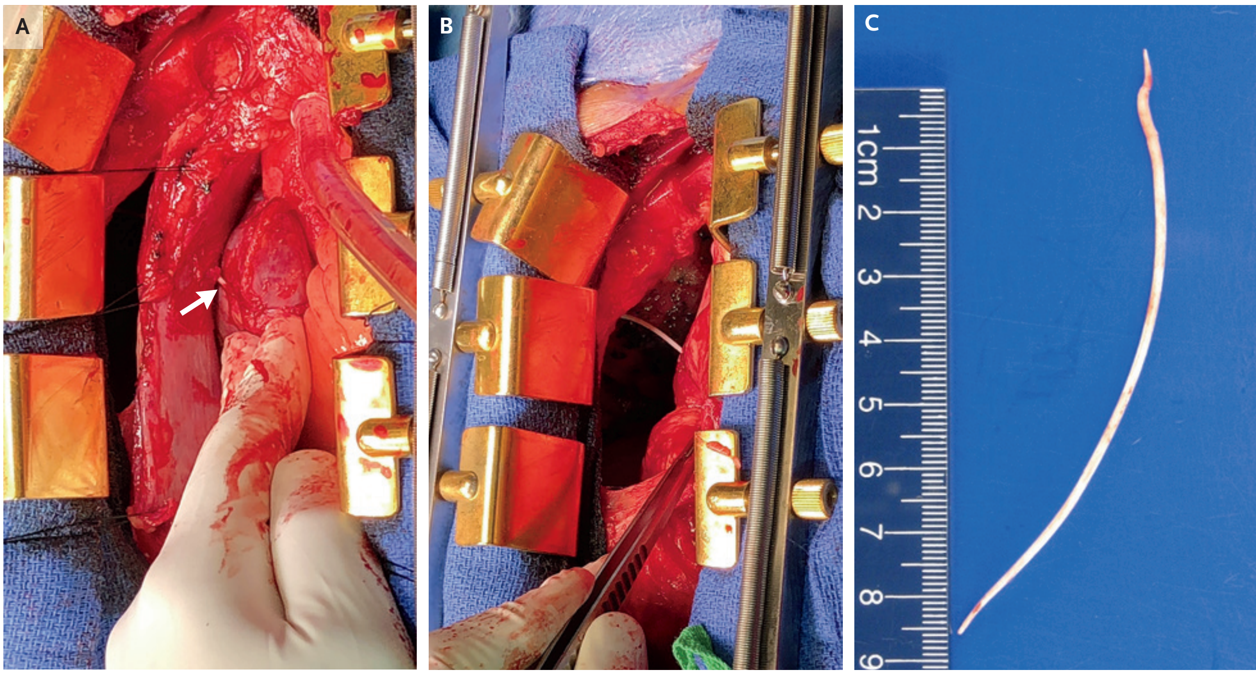
IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Intracardiac Cement Embolism



A 56-YEAR-OLD MAN WITH A RECENT VERTEBRAL COMPRESSION FRACTURE presented to the emergency department with a 2-day history of dyspnea and chest pain. One week before presentation, he had undergone an L5 kyphoplasty with polymethylmethacrylate medical cement. Five days later, he had sudden onset of pleuritic chest pain on the right side with radiation to the jaw and shoulder. At the current presentation, radiography and computed tomography of the chest revealed an intracardiac foreign body. The patient underwent emergency cardiothoracic surgery. During the procedure, a foreign body was found to be perforating the right atrium (Panel A, arrow), crossing the pericardium into the pleural space, and puncturing the right lung (Panel B). A sharp cement embolism measuring 10.1 cm in length and 0.2 cm in diameter was removed (Panel C and video). Cement embolism is a described complication of kyphoplasty; the cement can leak into the venous system, harden, and embolize. After the embolus was removed, the patient's right atrium was repaired. He had no postoperative complications, and at 1 month after the procedure he had nearly recovered.

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